

PROJECT APEX: MEDICAL CLINIC FIT-OUT

PHASE 5: ASSET COMMISSIONING, HANDOVER & CLOSEOUT CONTROLS

Phase 5: Commissioning & Practical Completion (APM Transition Stage)

The closing stage of the framework ensures a seamless transfer of the physical asset back to the client's operations team while locking down statutory, health and safety, and operational criteria.

Status: Practical Completion Achieved

Following 24 weeks of active construction execution, Project Apex was formally handed over on schedule. The £18,500 sub-floor variation was successfully absorbed, bringing final capital expenditure to **£603,500**—safely beneath the £650,000 ceiling.

1. Operational Handover Checklist

To safely sign off the JCT 2024 contract and transfer operational risk, the following governance checks were successfully executed:

Handover Stream	Verification Action Required	Status / Deliverable
Hygienic/M&E Systems	Pressure testing medical gas lines; balancing clinical HVAC airflow to ensure positive/negative pressure isolation zones.	Verified & Certified (NHS HTM Compliant)
Statutory File	Compilation of the building Health & Safety File, including building control approvals, structural warranties, and fire risk logs.	Delivered to Client Operations
Defect Identification	Comprehensive progressive site snagging walkthrough with the client. Logged 42 minor aesthetic snags.	95% Remediated; remaining items compiled onto a rolling Punch List.
Asset Transition	Delivery of digital As-Built drawings, Operation & Maintenance (O&M) manuals, and localized equipment training sessions.	Handed over to Facilities Manager

2. Post-Project Review & APM "Lessons Learned"

In accordance with the APM governance framework, a post-project review session was facilitated with key stakeholders to optimize delivery pipelines on future healthcare rollouts.

Key Takeaways & Operational Assets Developed:

- **Success — Agile Re-sequencing:** The decision to run parallel dry-lining paths while repairing the sub-floor slab rot in Week 6 completely preserved the critical path baseline. This highlights the immense value of inserting a flexible 2-to-4 week look-ahead window into the master JCT construction schedule.
- **Improvement Area — Utility Information Lag:** Hidden infrastructure mapping from older public assets introduces variance. Future clinic acquisitions should require a comprehensive sub-surface GPR (Ground Penetrating Radar) scan *prior* to final lease signing, rather than waiting for RIBA Stage 3/4.
- **Financial Performance Summary:** Contract base sum stayed completely preserved. The 10% structural contingency budget was drawn down by 28.4% to fix the wet rot, allowing **£46,500** to be returned directly to the client's commercial expansion fund at final accounting.